



Safe Haven: Access and the Provision of Care SOP

Scope

Site	Department, Division or Operational Area	Applicable to
Royal Perth Bentley Group (R PBG)	Mental Health (MH) Division	Peer Workers, Medical, Nursing, Allied Health

Purpose

This standard operational procedure (SOP) aims to provide guidance relating to the functions of Safe Haven – outlining criteria for attendance and processes to access the service and receive care.

Links to relevant documents within SOP

- [R PBG Safe Haven Model of Care and Service Plan](#) and [R PBG Policy Hub](#)
- [Escalation of care - Safe Haven Clinician Roles & Responsibilities Table](#)
- [Appendix I: Safe Haven Screening Tool](#)
- [Appendix II: Safe Haven Handover Sheet](#)

Overview of service

Safe Haven is a Royal Perth Hospital (RPH) Mental Health Category 4 and 5 Emergency department diversion program for up to 15 Attendees. Safe Haven provides psycho-social support and activities (e.g. Aromatherapy, stress balls) by Peer Workers and MH clinicians.

Location

N Block RPH on Victoria Square opposite St Mary's Cathedral, Perth

Definitions

Safe Haven	• Aims to: ○ Assist Attendees to improve, restore, and/or manage their wellbeing and mental health via support, safety planning and coping skills development, linkages/referrals to existing services, key workers, and other sources of support. • Provides: ○ Drinks and snacks, i.e. tea/coffee/ hot chocolate/ water/cold drink ○ Activities and de-stress items, e.g., aromatherapy, music therapy, art, board games, yoga, relaxing music, weighted blankets or pillows, sensory fish tank (no live fish). • Is a Peer Worker led service with Clinician support and oversight by a Clinical Nurse Specialist (CNS) and Nurse Unit Manager (NUM).
Clinician	CNS or Allied Health professional who is a skilled and experienced clinician with the ability to provide clinical assessment and oversight.
Peer Worker (PW)	Lived Experience Peer Workers, employed by a contracted agency. PWs deliver one to one support and group interventions to Attendees.
Safe Haven Team	Staff Composition 1. Peer Workers (two [2]) 2. Senior Clinician (one [1] – Allied Health Professional or CNS (one [1]) 3. 0.2 FTE NUM – managerial support 4. Additional clinical staff and/or security may be required at times. Hours of Operation 5. PWs or Clinician 1500 to 1945 hours on Friday – Sunday excluding public holidays. 6. NUM hours rostered 1 day/week.
Attendee	7. A person receiving services or care at Safe Haven.
Research Electronic Data Capture (RedCAP)	• A Department of Health (DoH) secure electronic data system to facilitate collection and analysis of Attendee details, including questions targeting several areas of brief assessment: ○ Reasons for the person attending ○ Knowledge of the service and what they hope to be able to undertake or receive at Safe Haven ○ Observational indicators will be undertaken to ensure that the person's needs can be accommodated.

Procedure details

Encouraging attendance

Safe Haven accepts Attendees via:

- **RPH ED diversion** for Category 4/5 MH patients
 - If a person is triaged or presents to the ED waiting room and willing to accept support for their MH distress/crisis/concern, the ED Clinician may refer the person to Safe Haven (Refer to **RPH Emergency Department: Triage and Waiting Room SOP** and **RPH Paediatric Presentations in the Emergency Department SOP**)
 - If the ED clinician assess the patient to meet Safe Haven inclusion criteria, contact the Safe Haven Clinician (via extension 43727) to determine current capacity, suitability and handover
 - Safe Haven clinician will attend ED to escort the patient to Safe Haven
 - It is the role of the Safe Haven clinician to actively engage with ED staff, provide accurate update of capacity and to assess ED presentations who may be potentially suitable for Safe Haven service
- **Self-presentation** i.e. based on a community recommendation (e.g. community mental health services [CMHS], hospitals/health services, non-government organisations [NGOs]) or walk in
 - Self-presenting attendees triaged by Safe Haven clinician congruent with ED Triage categorisation
- **Referrals from other community services** (e.g. CMHS, hospitals/health services, NGOs).
- It is the role of the PWs and contracted agency to create and maintain positive relationships with other community-based service providers. They are to ensure information relating to access of service/ supports are provided, and warm referrals to other agencies are continuously updated and distributed.
- Attendees accessing the service via this pathway will still be required to be triaged by a Safe Haven clinician.

Inclusions

Safe Haven is suitable for the following Attendees:

- Aged 18 years and above
- Experiencing MH distress/crisis (e.g. social isolation, exacerbation of MH/identified concerns) and require psychosocial support
- Experiencing suicidal ideation, without current plan with intent, and are willing to engage in peer support/safety planning and can guarantee their own immediate safety
- Reporting maladaptive coping strategies such as Non-Suicidal Self Injury but can confirm their immediate safety
- Assessed as medically suitable for attendance to Safe Haven by the referring clinician
- Presenting without acute drug or alcohol intoxication and who can guarantee their own safety, and the safety of others, within the immediate future.
- Acceptance of:
 - Safe Haven mutual obligation expectations, inclusion criteria, escalation of care and Service Halt Pamphlet information
 - Screening tool (RedCAP) prior to entry.

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Procedure details cont'd

Exclusions

Safe Haven is not suitable for the following Attendees:

- Aged under 18 years or minors attending with an adult
- Likely to require a psychiatric inpatient admission
- With significant historical and current aggression risk and/or behavioural disturbance
- Who are likely to experience acute deterioration (physiological and/or mental state) once accessing Safe Haven
- Presenting with acute drug intoxication or withdrawal
- Presenting with complex and/or unstable medical problems, requiring admission to a general hospital bed
- Involuntary inpatient according to the Mental Health Act (MHA) 2014 (e.g. Forms 1A, 3A, 6A, 6B)
- Experiencing suicidal ideation with current plan and intent
- Reporting self-harm or suicide ideation that requires immediate medical treatment and cannot guarantee their safety in the immediate future
- Who require a place to sleep
- Refuses service or uncooperative
- Who are unable to meet the mutually agreed upon expectations of appropriate engagement, as outlined in the Rights and Responsibilities, Exclusion Criteria, Escalation of Care and Service Halt Pamphlet given and explained on triage, whilst utilising the service
- Posing a risk of transmissible diseases (e.g. gastroenteritis, influenza-like illness)
- MH issues with the primary cause as homelessness
 - Information on available homeless services to be provided at triage, without entry to Safe Haven.

Note: Capacity for 15 Attendees may vary depending on PWs or staffing absence and inability to cover the shift.

The Safe Haven clinician is required to assess the environment and reduce the number of attendees in response to any perceived concern. This assessment is undertaken in partnership with the PWs. This will mean that the total Safe Haven Team Member to Attendee ratio will drop to ensure safety and confidence in service provision.

Procedure details cont'd

Roles and responsibilities of Safe Haven staff

Peer Workers:

- Orientate the Attendees to the space, providing education and guidance on safe sharing behaviours (i.e. Trauma Bonding) to minimise impact on other Attendees
- Share when appropriate their lived experience with intention
- Provide one to one intervention and engagement
- Plan and facilitate group-based activities
- Support Attendees to complete Safe Haven Safety/Wellness Plans
- Positively model and educate Attendees with strategies and coping skills to manage distress
 - Complete warm referrals to external agencies to support positive outcomes for Attendees
 - Work in collaboration with clinicians regarding decisions that affect safety and the operation of the Safe Haven
 - Work in alignment with the principles of practice outlined in the [WA Lived Experience \(Peer\) Workforces Framework](#)
 - Provide handover to external security contractors prior to commencement of duties regarding the expectations of their role.

Clinician:

- Completes Safe Haven Triage using RedCap Triage application for all Attendees
- Establishes and maintains collaborative practice for referrals, escalation of care and service updates with:
 - RPH Emergency Department NUM/Acting NUM/CNS/Triage
 - City East CMHS, Assessment and Treating Team (ATT) Program Manager or Team Leader
 - RPH Psychiatric Liaison Nurse (PLN) CNS
- Delivers clinical intervention as required
- Work in collaboration with the PWs regarding decisions that affect safety and the operation of the Safe Haven
- Escalates care to other clinical services as per Escalation of care - Safe Haven Clinician roles and responsibilities table
- Provide escort to and/or from the ED
- With collaboration from the PWs:
 - Provides one to one intervention
 - Supports Attendees to complete Safety/Wellness Plans
 - Positively model and educate Attendees with strategies and coping skills to manages distress
- RPH clinicians are required to work in alignment with their respective role JDF
- Provide handover to external security contractors prior to commencement of duties regarding the expectations of their role.

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Procedure details - Roles and responsibilities of Safe Haven staff cont'd

External Security Contractors

- If required, security contractors and/or increased clinical staffing may be utilised to temporarily meet service requirements
- External Security Contractors are to:
 - For guidance, refer to **Security Request for External Security Staff SOP**
 - Wear civilian clothing without “security” branding
 - Receive handover from the clinician and PWs of shift expectations - to occur prior to commencement of duties
 - Maintain open presence and visibility within the service
 - Take direction from clinicians or PW
 - Escort staff to their car or ED may be required at the direction of the clinician or PW
 - Any verbal de-escalation or talk down tactics to be guided by the clinician or PW
 - Any concerns surrounding role in Safe Haven to be raised and discussed with clinicians and PWs
 - Any concerns requiring escalation are to be made to the Security Contractors line management
 - No restraint of Attendees is permitted unless in a self-defence situation
 - Code Black/Duress protocol to be engaged as needed.

Attendance pathway

On each presentation Safe Haven Clinician is to:

- Complete brief screening of a consumer at the front door using the Safe Haven iPad. Select RedCAP logo or open web browser and enter the address: <https://datalibrary-rc.health.wa.gov.au>
- Use assigned username and password to enter the secure platform

Prior to commencing screening, a message reminder will appear on RedCAP to check the current occupancy of Safe Haven and the capped attendance numbers of 15. Complete a visual check of occupants and if full occupancy, suggest Attendee can return at a later time or offer alternative services/telephone supports.

If there are vacancies, explain and provide a copy of Rights and Responsibilities, Exclusion Criteria, Escalation of Care and Service Halt, and obtain verbal acceptance prior to Attendee entering Safe Haven. For further guidance, refer to section, '[Service Orientation](#)'

- Complete all screening questions. Refer to [Appendix I: Safe Haven Screening Tool](#)
 - Where an Attendee is suitable/not suitable to utilise Safe Haven (both generally and on specific occasions in the event of a regular Attendee) document assessment outcome and alternative referral into RedCAP

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Service orientation

The Safe Haven Clinician is to provide relevant information to Attendees ensuring information is delivered in a way that is understandable to the Attendee and that Attendee's agreement to the terms is sought.

- Advise that Safe Haven is an anonymous service and no medical records will be maintained, except where escalation of care is required, e.g. medical emergency, ED transfer, etc
 - Minimal records will be collected, and a copy offered or provided to the Attendee, if requested at a later date
 - Confidentiality will be maintained and Attendee anonymity preserved where possible, however:
 - In some circumstances, confidentiality may be breached in order to maintain the Attendee's safety and/or the safety of others. Or if the Attendee has allegedly engaged in criminal activity
 - There may be limitations to confidentiality and inadvertent disclosure of personal clinical information due to the shared environment
 - Staff will not share or disclose information to family/personal support persons (PSP) without Attendee's consent unless safety is at risk and/or a critical incident occurs (refer to [Escalation of care section](#))
 - PW/clinician to advise PW/clinician of potential personal relationship with Attendee
 - Attendee's information will not be shared or disclosed to other agencies or professional services without consent, i.e. if Attendee requests staff to talk to existing Care Coordinator or referrals to health/other services
 - That a contemporaneous record of how many people attend Safe Haven on specific dates will be maintained
- Information about Inclusion Criteria, Escalation of Care and Service Halt pamphlet. That includes:
 - Expectation of respectful language and boundaries to all other Attendees and staff that does not offend or upset others
 - Commitment to equal opportunities and no discrimination against any person on the basis of race, gender, sexuality, disability, age, religion or lifestyle
 - Non-smoking, no alcohol/drug/substance use or possession of illicit substances, alcohol/weapons, no sexual misconduct/ sexual harassment/ aggression/ violence, etc. Attendees should also be aware of the possible outcomes of inappropriate behaviours, for e.g. declining access to Safe Haven or police involvement
 - Maintaining other Attendee's confidentiality, including circumstances where Attendees know each other from other services, i.e., don't talk about another Attendee, or give personal information to other Attendees, particularly in communal areas, no photography or filming to be undertaken etc
 - No posting on any social media platform or via telephone/text messages about who is visiting Safe Haven or give any personal details of staff or other Attendees to protect people's right to privacy and ensure confidentiality
 - Escalation of care to ED may occur if the attendee presents with MH or physical health symptoms higher than scope of Safe Haven PW or clinicians
 - Service maybe halted and escalation to WAPOL if the Attendee:
 - Presents as violent or aggressive both physically or verbally towards staff, attendees, or property
 - Consumes alcohol or illicit substances
 - Engages in any illegal activity
 - At the discretion of the Safe Haven team.

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Service procedure

To promote staff safety and ensure Safe Haven service provision, PWs and Clinicians are to maintain competency in relevant safety skills:

- Attend RPBG Induction – PWs or new clinicians and Emergency Procedures orientation
- Complete training and competencies for PWs including de-escalation, self-care, and self-management skills. Refer to **Safe Haven Model of Care and Service Plan**
- Complete Safe Haven orientation including:
 - Familiarisation with **Safe Haven Model of Care and Service Plan** (including Team roles and responsibilities/service provision/facility design/management etc.)
 - Training in the use of Duress alarm – pendant and hardwire duress
 - Training in the use and entry of RedCAP and related measures for Clinicians
 - Local procedures – e.g., maintaining the ratio of one (1) Safe Haven team member to maximum five (5) Attendees with a total limit of fifteen (15) Attendees noting the Clinician in partnership with the PWs can reduce numbers were there is concern
- Complete annual mandatory training includes Aggression Prevention and Intervention [API] training for clinicians.

PWs / Clinician are to:

- Orientate Attendee to Safe Haven
- Encourage storage of personal property/items in the secure storage lockers to the rear of the Safe Haven site and collect at departure, as appropriate. Staff are required to supervise the placement of items. Searching of Attendees is not permitted on site but if there is any concern for an item in possession by an Attendee, the PW and/or Clinician can request the Attendee to place it in a locker and collect it on departure. The general principle for property is ensuring the safety of self and others and any item that poses a risk is to be placed in the locker storage areas made available. Each attendee has access to a lock for safe storage. Refer to **RPBG Patient Property Management Policy**
- Offer or discuss with new Attendees, where appropriate, information on:
 - Confidentiality of information
 - Safe Haven overview and provision e.g. brochure
 - Mutual respect/Mutual Accountability/expectations poster
 - Rights and Responsibilities, Inclusion Criteria, Escalation of Care and Service Halt Pamphlet/poster
 - Crisis card or Directory of useful numbers
 - Fire or emergency procedure
 - How Attendees can provide compliments and/or complaints to the service e.g. verbal feedback to the Safe Haven Team/EMHS electronic satisfaction survey. For further information on Attendee feedback refer to **Safe Haven Model of Care and Service Plan**.

Note: It is recognised that the Attendee may not take in all this information and may wish to take the information away or discuss it at a later time.

- Allow Attendee to proceed at their own pace and within an appropriate private space to foster trust and rapport.

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Service procedure cont'd

Intentional rounds

Between the PWs and Safe Haven clinicians are conducted at least hourly to identify:

- stressors or concerns
- ensure a welfare check and evaluate sense of safety at that time
- identify other MH care coordination support needs

This practise promotes communication to ensure safety and wellbeing updates between peers PWs and clinicians.

To prevent development of maladaptive coping strategies and the impact that may arise from sharing of trauma, staff will encourage and redirect Attendees to engage in group activities, rather than one-on-one Attendee interactions. Peer Workers can provide education and guidance around safe sharing with Attendees as part of the service orientation and revisit as required. Clinicians and PWs will be available to provide support, attentive listening, and guidance, ensuring the wellbeing of everyone involved.

This practice encourages communication to ensure that safety and wellbeing updates are shared between PWs and clinicians.

Pre-briefing and debriefing

Pre-Briefing:

- Conducted at the start of every shift, before service commences at 1500hrs, and facilitated by the Clinician
- To give PW and the Clinician opportunity to discuss staff wellbeing
- To promote team building
- To reinforce positive communication and a culture of continuous improvement.

Debriefing:

- Conducted at the end of each shift
- To discuss shift actions and processes and promote self-reflection
- To reinforce team building and a culture of continuous improvement
 - What went well?
 - What did we miss?
 - What can we do differently?

Service procedure cont'd

Available support / service

PWs and Clinicians are to offer Attendees:

- One-to-one support
- Social time (i.e. Attendees can sit with a drink and snack or engage in conversation with other Attendee's/people waiting)
- Problem solving, de-escalation and other psychosocial support, as required
- Access to a low stimulus quiet space; provided with minimal with soft furnishings provided to promote de-escalation and, if suitable, engage in mindfulness activities etc
- Access to self-regulating activities - individual or group based (i.e. yoga, music, drawing, art, etc.)
- Referrals or support to engage with other health services and access support, i.e. CMHS, general practitioner (GP), other services, if appropriate
- Support plan development (i.e. Safe Haven Wellness Plan) that includes use of the wider network of health and care services, where relevant. Wellness plan is to include:
 - Early warning signs
 - Indicators which show the Attendee is coping well
 - Strategies to promote Attendees wellness/access support, including for the Attendee, significant other/PSP, i.e. reduce risk and enhance safety.

PWs are to advise the Clinician of:

- Identified safety or risk concerns (e.g. current suicidal ideation with plan and intent)
- Required follow-up
- Escalation of care
- Any challenges the PW believes to be outside their level of training or experience.

Working Collaboratively

The Safe Haven team, PWs and clinicians will engage activities the Safe Haven service based on consistent approaches.

- Shift by Shift Handover between workers
- End of Weekend handover to team leads
- Fortnightly RPH/RUAH team leads catchup
- Monthly team meetings including all.

There will be opportunity for debrief as a team and to revisit rules of engagement when breaches occur, including:

- Threats of violence or aggression by the Attendee to others
- Any incident by the Attendee necessitating the activation of the duress alarm
- Any other risk issues that arise.

Should a critical incident occur at the Safe Haven (i.e. an unexpected, sudden and traumatic event, outside of the usual range of daily experiences involving a personal threat, which evokes extreme stress, fear or injury and affects one's sense of security), PWs and/or Clinicians are to

- Trigger the escalation of care pathway for the Attendee as appropriate (refer to Escalation of Care section below)
- Offer brief, individual informal defusing sessions to affected Attendees at the time of the incident.

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Service procedure – Available support/service cont'd

Further defusing sessions are not appropriate and will require referral to counselling or support services (e.g. Allied Health).

- **For Attendee debriefing and support:** refer to **Mental Health Consumer / Carer /Visitor Support Post-Critical Incident SOP**
- **For Staff debriefing and support:** refer to **EMHS Critical Incident Stress Debriefing**

Note: PWs are provided monthly debriefing sessions by RUAH Community Services and have access to Ruah operational supervisor On Call and Peer Lead On Call.

Should incidents occur involving Attendees' breakage, loss, alleged theft and other claims refer to the **EMHS Security: Theft and Loss Reporting Policy** and **RPBG Patient Property Management Policy**.

Verbally Handover Information to the next shift i.e. issues discussed, frequent Attendees, etc. verbally and using the 'Safe Haven Handover Sheet' ([Appendix II](#)).

Escalation of care

- Escalation of an Attendee's care is to occur after the Clinician has assessed the situation(s) or the Attendee's mental state and/or physical health and risk level following a reasonable attempt to collaborate with and engage the Attendee and/or their family/PSP (as appropriate).
- Clinicians are to exercise their clinical judgement within their role, acting more as a Manager of the Safe Haven service. They have the authority to determine the maximum number of attendees in the space based on current Attendees utilising the service, subjective concerns, and their confidence in the Safe Haven team. PWs will refer to the Clinician and may complete handover. Clinicians hold ultimate responsibility for the care and safety of all Attendees and staff.
- Triggers for the escalation of care for an Attendee are as follows:
 - Any critical incident including death, sexual assault and/or physical violence resulting in serious injury to an Attendee and/or staff
 - Any complex presentation where contradicting views to those of the Clinician or PW about Attendee's best interest are expressed, which makes it difficult to arrive at a robust and reliable clinical formulation
 - Any mental state and/or physical health deterioration or medical emergency
 - Any significant threats of assault or of a legal nature made by Attendee against RPBG staff and/or other(s) that might lead to legal implications (e.g. litigious Attendees who may be dissatisfied with their care) unable to be resolved by the Clinician
 - Any challenges the assessing Clinician believes to be outside their scope of practice or clinical experience.
- In circumstances where an Attendee requires escalation of care, for further guidance refer to the '[Escalation of care - Safe Haven Clinician Roles & Responsibilities Table](#).'

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Escalation of care - Safe Haven Clinician roles & responsibilities table

Criteria	Hierarchy	Escalation of Care
Physiological concern or deterioration		
Difficulty breathing Respiration Rate (RR) ≤ 4 or ≥ 36 / min	Emergency Call	Clinician is to: Dial Code Blue '55' *If assistance with transfer onto a trolley is required a Hovermatt request on Computerised Assisted Response Program (CARPs) can be made. Escort Attendee to ED in conjunction with PW whilst still maintaining the 1:5 ratio Notify the PW, Attendee's family/PSP of ED transfer, as appropriate Refer to the RPBG MET Clinical Practice Standard (CPS)
Collapse		
Loss of Consciousness		
Seizure		
Chest pain		
New pain		
Uncontrolled pain		
Respiratory symptoms (fast, slow, irregular or increased work of breathing)	Clinical (MO) Review	Clinician is to: Contact Safety All Hours For Everyone Teams (SAFE) via CARPs RPH Emergency Physician in Charge (EPiC) [Tel: (08) 9224 1676] Clinician/PW to encourage Attendee to seek out GP/ call HealthDirect or other health services, referral to SAFE where concerns are raised
Sweating more than normal		
Dizziness		
Nausea		
Vomiting		
Injury to the body requiring greater than general first aid care	Senior Nurse Review	Clinician is to: Contact SAFE Team
Subjective concern (attendee, staff member or other for the person)		
Attendee voiced concern for physical health/physical health complaint	Non-Emergency	Clinician/PW to encourage Attendee to seek out GP/ call HealthDirect/ other health services

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Mental Health and/or Cognitive Concern	Hierarchy	Escalation of Care
*Applies to internal to Safe Haven only. If Attendee leaves the building, referral to WAPOL is to be considered and Handover to ED/CMHT		
Changes (deterioration) in the person’s behaviour, cognitive function, perception, emotional or mental state	Safe Haven Clinician is to: - De-escalate - MSE in triage form including formulation and plan Direction of escalation 	1. Recommend Attendee to go to the Emergency Department for assistance – call ED Triage/ RPH ED Psychiatric Registrar Tel: 0404 894 037 for assessment - Encourage Attendee to walk to ED and be accompanied by a clinician (i.e. ED Psychiatric Registrar/Safe Haven Clinician or Psychiatric Liaison Nurse (PLN) or alternative transportation, e.g. via St John’s Ambulance, Wilson Medic One, etc. as per EMHS Mental Health Transportation Policy
Threats of violence or actual violence		2. Call PLN or Mental Health Emergency Response Line (MHERL) to request City East ATT to complete assessment in Safe Haven
Threats of self-harm or actual self-harm		3. Activate Duress/ Dial ‘55’ 4. Call Emergency services (e.g. police, ambulance)
Threats of weapons		
Illicit substance possession		
Subjective concern (attendee, staff member or other)		
Safety measures and awareness of support on exit from Safe Haven	Clinician/PW to advise Attendees of: MHERL City East Assessment Treatment Team (ATT) after hours	
Where escalation of care is required: Documentation - Complete relevant documentation including: - For Mental Health Division (MHD) Attendees, complete/update current: - Triage form and Risk Assessment Management Plan (RAMP) on WebPSOLIS as appropriate - Refer to Department of Health Statewide Standardised Clinical Documentation (SSCD) for documentation requirements - Document Mental State on WebPSOLIS for Attendees provided with support by the Safe Haven Clinician. Note all planning and decision making to support best practice.		

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Threats of violence or actual violence

If the Attendee presents with current aggression risk and/or behavioural disturbance Clinician is to mitigate safety/security for all, including by:

- Accessing alerts for existing mental health patients on PSOLIS and/or for General Health, refer to risk flags added to iSOFT for inpatients and/or follow-up with CMHS
- Considering use of the video intercom, i.e. complete screening and making contact with unknown persons via the Closed Circuit Television (CCTV) monitors to determine what is occurring outside of the building, if there are any indicators for risk (to the individual or others) and respond over the intercom with an introduction, prior to allowing entry
 - If required to stop an individual's entry into Safe Haven due to risk/safety concerns, consider shut down of service/set up. **RPBG CCTV Monitoring in Mental Health Inpatient Areas SOP**
- Considering use of Security (contact via [Security Hub](#)). Refer to **EMHS Security Policy/ RPBG Security Request for External Security Staff SOP**
- De-escalating strategies. For guidance, refer to **Prevention and Management of Aggressive Behaviours within Mental Health Inpatient Setting SOP**
- If safety/risk concerns have not resolved (e.g. shouting, swearing, threats to self and/or others, destruction of property etc), notifying RPH Security on ext. 41738/42811 mobile 0435 311 656 and/or activate the duress system, i.e. fixed hard wired (located at the front door, bathroom entry wall and kitchen area of Safe Haven). Refer to **RPH Emergency Procedures** and **EMHS Duress Alarm Systems Policy**
- Activating a Code Black '55' in accord with **RPH Emergency Procedures**, specifically pertaining to Safe Haven
- Contacting emergency services (e.g. police, ambulance) via mobile, Phone 000.

Critical Incident Reporting

Clinician or PW are to:

- Inform the Attendees' family or PSP if the Attendee's safety is at risk and/or a critical incident has occurred, as appropriate
- **In the event of a serious illness or injury to an Attendee or staff member**, seek assistance including transfer to ED, particularly with management of serious traumatic injuries and ensure the person is medically cleared prior to returning to the Safe Haven if required
 - NUM/Clinician is to notify staff member's family of the incident if transfer to ED for medical care is required, with staff member's consent, where relevant
 - Provide details on how staff can access the Employee Assistance Program, where required
- Complete Datix Clinical Incident Management System (CIMS) and/or Combined Hazard or Injury Report (CHoIR)

In the event of an emergency situation (e.g. fire, flood etc.) the Clinician is the Area Warden. Staff are to raise the fire alarm or activate the fixed hard wired /mobile duress alarms. Staff are to escort Attendees to the designated Emergency Assembly point, and ensure that no Attendees re-enter the building until it is safe. Refer to **EMHS Emergency Disaster Management Policy**.

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Compliance monitoring

Compliance will be monitored as part of the:

- Datix CIMS system process and CHoIR (e.g. clinical incidents related to Safe Haven)
- Compliments/Complaints Management System
- Monthly reporting of key performance indicators (KPIs)/outcome measures for Safe Haven to the MHD safety quality meetings/EMHS. Refer to **R PBG Safe Haven Model of Care and Service Plan**
- Facilities Management/Security monthly audits to the MHD /EMHS, including checks that hardwire duress and CCTV are in working order.

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Aboriginal Health Strategy Team, EMHS

Aboriginal Health Impact Statement and Declaration

The voice of the Aboriginal community is reflected in the Safe Haven: Access and the Provision of Care SOP through consultation with the Aboriginal Health Strategy Team and the Aboriginal Workforce Engagement Group to ensure that cultural appropriateness and the health impacts on Aboriginal people have been considered and incorporated (ISD Record 1565).

Note: Within Western Australia, the term Aboriginal is used in preference to Aboriginal and Torres Strait Islander, in recognition that Aboriginal people are the original inhabitants of Western Australia. No disrespect is intended to our Torres Strait Islander colleagues and community.

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Related policies, practice standards, clinical guidelines

RPBG Policy Hub

- [Clinical Care of People Who May Be Suicidal Policy](#)
- [Medical Emergency and Cardiorespiratory Arrest Management \(MET\) CPS](#)
- [CCTV Monitoring in Mental Health Areas SOP](#)
- [Infection Control Manual](#)
- [Respiratory Virus Infection Prevention and Management Clinical Guideline](#)
- [Clinical Handover Policy](#)
- [Sexual Safety Policy](#)
- [ED: Triage and Waiting Room SOP](#)
- [Paediatric Presentations in ED SOP](#)
- [Security Request for External Security Staff SOP](#)
- [Patient Property Management Policy](#)
- [Mental Health Patient, Carer and Visitor Support Post-Critical Incident SOP](#)
- [RPBG Safe Haven Model of Care and Service Plan](#)
- [Prevention and Management of Aggressive Behaviours within Mental Health Inpatient Setting SOP](#)
- [RPH Emergency Procedures](#)

EMHS Policy Hub

- [Prevention and Management of Workplace Aggression and Violence Policy](#)
- [Acute Response Mental Health Emergencies/Crisis Policy](#)
- [Alcohol and Prohibited Substances Policy](#)
- [Critical Incident Stress Debriefing](#)
- [Security: Theft and Loss Reporting Policy](#)
- [Mental Health Transportation Policy](#)
- [Security Policy](#)
- [Duress Alarm Systems Policy](#)
- [Emergency Disaster Management Policy.](#)

Department of Health Policy Hub

- [Statewide Standardised Clinical Documentation Policy](#)
- [Safety Planning for Mental Health Consumers Policy](#)
- [Principles and Best Practice for the Care of People Who May Be Suicidal](#)
- [Principles and Best Practice for the Care of People Who May Be at Risk of Exhibiting Violent or Aggressive Behaviour](#)

Other

- [Safe haven HUB](#)
- [Mental Health Act 2014](#)
- [Chief Psychiatrist's Standards for Clinical Care](#)
- [Charter of Mental Health Care Principles \(Mental Health Act 2014\) - WA Department of Health](#)

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Legislative requirements, the evidence and the standard

ACSQHC NSQHS Standards 2nd Edition (2021):

Standard 2: Partnering with consumers

Standard 6: Communicating for Safety

Standard 8: Recognising and Responding to Acute Deterioration

NSMHS Standards (2010):

Standard 2: Safety

Standard 6: Patients

Standard 10: Delivery of care

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Appendix I: Safe Haven Screening Tool

1. Safe Haven (SH) Screening Questions

The Safe Haven Clinician will administer the screening questions each Attendee prior to entry into the Safe Haven and document their responses on the RedCAP platform

2. Safe Haven Criteria

Inclusion

- An Attendee is defined as a person who is warmly welcomed into Safe Haven with the intention of engaging with Peer Support Workers
- An Attendee acknowledges and upholds the mutual expectations of Right and Responsibilities at Safe Haven
- An Attendee participates in completing the screening tool (RedCAP) prior to entry, to assess their suitability for Safe Haven.

Exclusion

- Individuals presenting with or escalating to:
 - Experiencing acute intoxication, under the influence of substances; or consumes drugs or alcohol
 - Displaying aggressive or any forms of violent behaviours towards other attendees, staff, or property
 - Multifaceted or unmanageable medical problems
 - Unwilling to engage with Peer Support Workers
 - Presence of an infectious illness
 - Failure to uphold the Rights and Responsibilities of the Safe Haven
 - At the discretion of the Safe Haven team

1. Is the Safe Haven at capacity? e.g. 15 persons

☐ Yes – person advised to try again in 30-60 minutes

☐ No – proceed with questions below

Introduction

Hi, I am "NAME" Welcome to Safe Haven

Safe Haven is a space for people to have an opportunity to meet with a Peer Worker and to share their story as an alternative to going to the Emergency Department.

The Safe Haven can provide:

- a) A listening ear for you to talk to someone
- b) A safe place to sit or participate in an activity
- c) Help or assistance to manage any current concerns.

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Appendix I: Safe Haven Screening Tool cont'd

2. What is your name? Click or tap here to enter text.

Date:

Time:

3. COVID Screen

- Do you have any of the following symptoms: Fever, chills, cough, shortness of breath or sore throat?

☐ Yes – Not suitable for entry. Perform or offer RAT

☐ No – Proceed with the next question

4. What brings you here today?

Click or tap here to enter text.

5. Where did you hear about this place? (SH)

Click or tap here to enter text.

I have a few quick questions that I need to ask before I can welcome you in today. They will not take too long. Need for this?

6A. Have you drunk any alcohol or taken any substances today?

☐ No

☐ Yes

WHAT? Click or tap here to enter text.

HOW MUCH? Click or tap here to enter text.

WHEN? Click or tap here to enter text.

CLINICIAN TO ASSESS AND MARK:

Appendix I: Safe Haven Screening Tool cont'd

6B. Is there any evidence of the following? Tick those that apply

- ☐ Slurred Speech
- ☐ Pin-point pupils
- ☐ Injury to self
- ☐ Smell of ethyl alcohol, or ethanol (ETOH)
- ☐ Smell of Cannabis
- ☐ Loss of balance
- ☐ Drowsiness
- ☐ Vision problems
- ☐ Loss of memory or judgement
- ☐ Signs of Euphoria such as – delayed reaction time, decrease inhibitions, increased chattiness, and confidence suggestive of drug or alcohol use?
- ☐ Other-----

6C: Is referral/escort to the Emergency Department required?

- ☐ Yes
- ☐ No

CLINICIAN TO ANSWER THE FOLLOWING:

Tick the box for Yes.

7. Do you have any concern based on visual observations, for this person entering Safe Haven?

- ☐ No
- ☐ Yes – what: (E.g. history) [Click or tap here to enter text.](#)

Kindly inform the individual that they are unable to enter at this moment and suggest revisiting on another day.

Appendix I: Safe Haven Screening Tool cont'd

8. Advise the person that the Safe Haven operates on a Mutual Accountability:

Where we expect the best behaviour from Attendees in return for treating Attendees with safe and quality care.

We ALL agree to:

- No violence of any kind, including threats to others, swearing, aggressive language, hitting or throwing things
- No racism, hurtful or offensive remarks
- Empower and encourage each other to take ownership and make good choices
- No smoking, use of alcohol or any illicit substances, prescribed or non-prescribed medications inside the Safe Haven.

9. Do you (the attendee) accept this?

☐ No – unable to enter. Say thank you and unable to enter until this is agreed and perhaps try again another day.

Staff can also contact Security directly on extension 42811 or page 2121/dial 55/activate duress if required

☐ Yes – able to enter.

Date: Click or tap here to enter text.

Time: Click or tap here to enter text.

Appendix II: Safe Haven Handover Sheet



Government of Western Australia
East Metropolitan Health Service

Royal Perth Bentley Group

Safe Haven Information to handover to the next shift

Date	
Staff Member Name	
Accessed from safe	☐ Yes ☐ No
Filed in safe	☐ Yes ☐ No

Information to handover

Signed: Date:

Safe Haven Handover Sheets are accessed in an A4 Book/Binder and filed /stored securely in the safe.

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